



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متكرر

إسم المريض ربيعاً : شوقيه نصار لوزير فهميم
المحافظة : القاهرة
الحى / المدينة : منشيه ناصر
العنوان : -
رقم البطاقة : 27309172100701 رقم التليفون : 01223385368
تاريخ تسجيل الإستمارة : 26-09-2026
العمر (تاريخ الميلاد) : 17-09-1973 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----) المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
الوظيفة: -----
الوظيفة: -----
التوقيع: -----
التوقيع: -----
التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقرر اللجنة بان البيانات المدونة فى هذا النموذج صحيحة.
تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض
بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- د. / ----- د. / ----- د. / -----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : -----
قسم : -----
جهة العلاج : -----
نفقات : -----
ملاحظات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

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Name: شوقيه نصار لويز فيهم National ID: 27309172100701	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

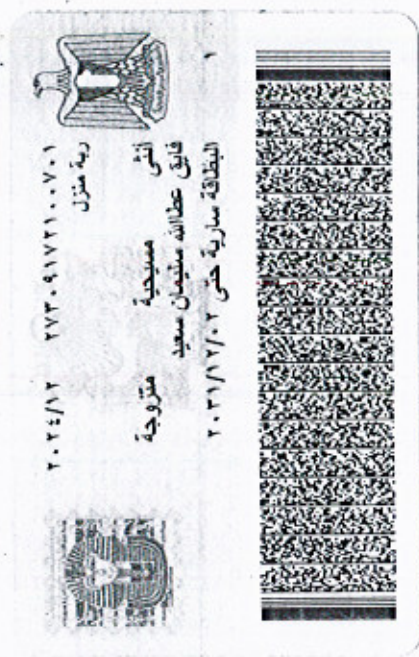
نتشرف بان نوضح فيما يلي تقريرنا عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



MDT FORM

5/1 / 2026 G

Name	فؤاد لوز	MRN	414 / 2025
Age	50	ID	
Menopausal status	Post	Phone	01279092836
NRS SCORE		Marital status	Married
BMI		Family history	None
CO morbidity	M-free	REASON OF MDT	Post op.

Clinical Data

BX: IDCT II				R L TNM
ER: 8/8	PR: 8/8	HER 2: 3+	Ki67:	
Sonomammography: Post MACT -				
Mets Workup: ACX4 taxan phygo x4				

Recommendations:
Then 1st SM + SLNB ->
free margin
(1.5cm) 0.5cm
ne 1.5cm

MDT OUTCOME & PLAN

- PC12
For - PORT - Ado. Hcrt 5x X14.
- A.I x7

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
M. Kell		Ne R 5/12/2025			

Women's Health Outreach Program

Name: shawkia nassar lovai

Date: 16-2-2025

Contrast Enhanced Digital Mammography

Clinical summary:

A Female patient recently diagnosed IDC grade II , with reteroareolar DCIS, for CESM assessment

Technique

MLO and CC views (low energy & recombined post contrast)

Targeted High resolution ultrasound

Detailed Findings:

Density: ACR c

Background Uptake: minimal

Right breast

- Low energy (non contrast images) show : no masses, asymmetry, distortion or suspicious calcifications.
- Recombined (post contrast) images show: no pathological uptake.
- Ultrasound demonstrated no solid or cystic masses.
- Normal skin/subcutaneous tissue and nipple/areolar complex.
- Non-specific axillary LNs.

Left breast

- Low energy (non contrast images) show : scattered microcalcifications seen almost at UOQ ,central and inner lower quadrants .
- A ill-defined density seen at UOQ with small foci of microcalcifications some of which is seen clumped and the nearest microcalcification is about 1 cm away from nipple .
- Recombined (post contrast) images show: a suspicious faint contrast uptake seen at the UOQ (Corresponding to the related ill defined density)
- Also noted non mass like enhancement seen involving the UOQ , down to the lower inner quadrant (corresponding to the distribution of the microcalcifications)
- Ultrasound demonstrated ill defined hypoechoic defined mass lesion with curvilinear retroareolar extension
- Normal skin/subcutaneous tissue and nipple/areolar complex.
- Non-specific axillary LNs

Patient Name: Shawkeya Nassar Lowiz Fahim	Date : 27/07/2025
Exam : C.T CHEST	Age : 52 Year(s)
Medical ID: 414/2025	Pat. ID: 97562

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, :-

Reported case of breast cancer

I Chest:-

- No mediastinal or hilar enlarged L. Ns.
- Small nodule is seen at superior segment lingular lobe measures 9mm.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average size cardiac size. No pericardial effusion detected.
- Normal appearance of the chest wall.
- **II Abdomen & pelvis:**
- The liver is mild enlarged in size with homogenous CT texture. No focal lesions
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, and both suprarenals.

III.Scanned skeleton:

- No worrisome lytic or sclerotic bony lesions.

Opinion:

- lingula solitary nodule for follow up

DR/Reham Khedr,,MD

27/7/2025

Patient Name: Shawkeya Nassar Lowiz Fahim	Date : 21/08/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 52 Year(s)
Medical ID: 414/2025	Pat. ID: 97562

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: Left pathologically proven IDC and DCIS, post NACT assessment study.

Findings:

Right breast.

- Heterogeneously dense breast parenchyma that may obscure small lesion. (ACR c)
- No speculations, masses asymmetry, distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated two circumscribed oval parallel homogenous hypoechoic benign looking solid mass lesions with absent internal vascularity on color Doppler study seen at 12-1 o'clock measuring 9 x 7 mm & 12 o'clock measuring 7.5 x 5 mm.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Left breast.

- Heterogeneously dense breast parenchyma that may obscure small lesion. (ACR c)
- UOQ, LIQ middle and inner thirds multiple clustered coarse heterogenous microcalcifications reaching the nipple with surrounding distortion.
- **Ultrasound** demonstrated UOQ and LIQ areas of distortion surrounding multiple prominent ducts seen reaching the nipple with soft tissue echogenicity and calcifications within. No suspicious masses detected.
- 2-3 o'clock, middle third, circumscribed oval parallel homogenous hypoechoic benign looking solid mass lesion with absent internal vascularity on color Doppler study seen at 2-3 o'clock measuring 9 x 6 mm
- Normal nipple/areola complex.
- Non-specific axillary LNs. No pathological infra or supra-clavicular lymph nodes.

Conclusion:-

Right breast (BIRADS 3): Two probably benign solid mass lesions mostly fibroadenomata.

Left breast (BIRADS 6): complete radiological response of the previously reported malignant masses. Still noted suspicious microcalcifications and distortion. UOQ probably benign solid mass lesion mostly fibroadenoma.

Best Regards,
Dr. Amal Hegazy, M.Sc.

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with recommendations:

Category & recommendations

BIRADS 0: Non-informative study.

BIRADS 1: Normal study, routine screening

BIRADS 2: Benign findings, routine screening

BIRADS 3: Probably benign, for close follow up.

BIRADS 4: Probably malignant, for biopsy.

A: mild suspicion.

B: moderate suspicion.

C: high suspicion.

BIRADS 5: Malignant, for biopsy.

BIRADS 6: Pathologically proven malignant

Signature Dr. Amal Hegazy

PM 1:42 2025/08/21

Suspected Cancer Risk

0 %

0 %

2 %

50 %

98-100%



Patient Name	الأستاذة / شوقيه نصار لؤي شيمس	Request No.	: 13000151
Patient No.	: 27309172100701	Request Date	: 31/12/2024
Path No.	: 5268/24	Verification Date	: 29/01/2025
Sex	: Female	Financial Category	: نفقة الدولة - اورام دار السلام
Age	: 51 Y	Department	: Out Patient
Referring Doc :		Registered by	: Mrs. Mayada Hegazy

Pathology Report

Clinical Data: Left breast masses.
Nature of Specimen: Tru-cut biopsy.

Gross Examination:

Two specimens:

1-Left breast highly suspicious mass:

Five grayish white tissue cores up to 1 cm, totally submitted.

2-Para areolar dilated ducts with microcalcification:

Fragmented grayish white tissue cores up to 0.7 cm, totally submitted.

- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination:

1-Left breast highly suspicious mass:

* Histological type: Invasive duct carcinoma.

* Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.

* Ductal carcinoma in situ: No.

* Perineural invasion: No.

* Angioinvasion: No.

* Microcalcifications: Undetected.

* IPT (Positive control showed appropriate reactivity):

- ER: Strong nuclear reaction in 80% of tumor cells (score 8/8).

- PR: Strong nuclear reaction in 80% of tumor cells (score 8/8).

- Her2: Strong complete membrane reaction in >10% of tumor cells (score 3+).

- Ki67: 22% nuclear reaction of tumor cells.

2-Para areolar dilated ducts with microcalcification:

* Histological type: Ductal carcinoma in situ DCIS grade II (solid pattern), with intraductal

د. طارق عبد الحليم
ف. ح. ص. ح.

Verified by :

Dr. Toka Medhat

Printing Date : 05/02/2025 09:39:51

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Printing By : marwa mohamed youss



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name : الأستاذ / شوقيه نصار أبويز نجيم
Patient No. : 27309172100701
Path No. : 5268/24
Sex : Female
Age : 51 Y
Referring Doc :

Request No. : 13000151
Request Date : 31/12/2024
Verification Date : 29/01/2025
Financial Category : نفقة الدولة - اورام دار السلام
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

papilloma.

- * Perineural invasion: No.
- * Angioinvasion: No.
- * Microcalcifications: Undetected.

Diagnosis:

Left breast masses, Tru-cut biopsy:
1-Left breast highly suspicious mass:
Invasive duct carcinoma, Grade II.

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 8/8).
- Her2: Positive (score 3+).
- Ki67: Positive in 22% of tumor cells.

2-Para areolar dilated ducts with microcalcification:

Ductal carcinoma in situ DCIS grade II (solid pattern), with intraductal papilloma.

Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and

Verified by :

Dr.Toka Medhat

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Patient Name : الأستاذ / شوقيه نصار لويز فهميم
Patient No. : 27309172100701
Path No. : 3482/25
Sex : Female
Age : 52 Y
Referring Doc :

Request No. : 13003195
Request Date : 12/11/2025
Verification Date : 06/12/2025
Financial Category : جوستاف روسي - نفقة الدولة
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left breast cancer, Post-NACT.

Nature Of Specimen: Left simple mastectomy & Sentinel lymph nodes biopsy.

Gross Examination:

- * Left simple mastectomy specimen measured 20x20x5 cm, with covering skin ellipse measured 19x12 cm. Nipple & areola looked unremarkable grossly.
- * Serialing showed a diffuse central tumor bed site 6x5.5x4 cm, located 0.8 from the least deep margin.
- * Sentinel lymph nodes: Multiple fibrofatty tissue pieces collectively measured 5x5 cm, entangling 5 lymph nodes, the largest is 3.5x1.5x1 cm.
- * 9 Blocks:
1:6- T.B.S., 7- Deep margin., 8- N&A., 9- ALNs.
- Gross was done by; Dr. Mennan Hossam.

Microscopic Examination:

- * Histological type: No residual viable remnants of the previously diagnosed invasive tumor.
- Cellularity (0%).
- * Angioinvasion: Absent.
- * Ductal carcinoma in situ: Absent.
- * Additional findings: Effect of therapy.
- * Nipple & areola: Free.
- * Deep margin: Free.
- * Sentinel lymph nodes: Five lymph nodes free of metastatic deposits (0/5).
- * Histologic response to therapy: Pathological complete response to therapy.

Diagnosis:

Left breast cancer (Post-NACT), Left simple mastectomy & Sentinel lymph nodes biopsy:
- No residual viable remnants of the previously diagnosed invasive tumor.

Verified by :
Dr.Duaa Mohamed

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Patient Name : الأستاذ / شوقيه نصار لويز فهميم
Patient No. : 27309172100701
Path No. : 3482/25
Sex : Female
Age : 52 Y
Referring Doc :

Request No. : 13003195
Request Date : 12/11/2025
Verification Date : 06/12/2025
Financial Category : جوستاف روسي - نفقة الدولة
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

- Ductal carcinoma in situ: Absent.
- Nipple & areola: Free.
- Deep Margin: Free.
- Sentinel lymph nodes: Five lymph nodes free of metastatic deposits (0/5).
- Histologic response to therapy: Pathological complete response to therapy.
- Stage: ypT0, ypN0 (sln).

Signature
Dr. Walaa Ibrahim Ghoneim.
Dr. Heba El-Shafeey.

Verified by :
Dr.Duaa Mohamed

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